BASELINE → TREATMENT → FOLLOW-UP

Checks through the whole course

The columns show the order of care, not equal time intervals. Extra tests depend on symptoms, results and the consultant’s plan.

Check	Before starting	Days 1–3 of each cycle	Between visits	Course / response review	After chemotherapy
Blood counts	Baseline blood count	Review results before treatment	Further counts if symptoms or clinical plan require	Review infection, bleeding and previous toxicity	Follow unresolved low counts
Kidneys, liver and fluids	Renal/liver profile; verify clearance and dose suitability	Renal/liver tests before each cycle; hydration/electrolytes with cisplatin	Repeat sooner after dehydration or illness	Review organ function before continuation or modification	Follow kidney/electrolyte/liver abnormalities
Hearing and nerves	Record baseline symptoms; audiology if indicated	Review symptoms before each cycle	Report tinnitus, hearing loss, numbness or loss of function	Balance benefit and lasting toxicity	Review persistent changes
Prevention and route	Confirm infusion or capsule plan and a written multi-day anti-sickness schedule	Check support before each treatment day; pharmacy confirms oral doses	Follow support through two days after last SACT; do not extend medicines yourself	Review interactions and support after any dose/route change	Confirm which support medicines stop or continue
Disease assessment	Confirm extensive-stage SCLC and individual treatment aim	Assess symptoms and fitness	Extra assessment for new concerns	Scans/tests as directed by consultant; no universal scan interval	Agree follow-up or a separate next treatment plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00280 v7 · official regimen · NCCP NCIS Medical Oncology antiemetics V8, Table 1 and multi-day guidance · Manufacturer cisplatin product information

THE WHOLE TREATMENT JOURNEY

Cisplatin + Etoposide: treatment and checks

Extensive-stage small-cell lung cancer · chemotherapy

Cisplatin is planned on Day 1. Etoposide is planned on Days 1–3: usually all three days by infusion, or a prescribed alternative with an infusion on Day 1 and capsules twice daily on Days 2–3. The source plans 4–6 cycles, usually every 21 days. The team may delay, reduce or stop treatment according to your health and response.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Days 1–3 of each cycle	Between visits	Course / response review	After chemotherapy
Chemotherapy course NCCP 00280	Agree indication, route and course	Day 1 Cisplatin + etoposide; etoposide continues Days 2–3	Recovery and symptom monitoring	The source plans 4–6 cycles, usually every 21 days. The team may delay, reduce or stop treatment according to your health and response.	Follow-up or a separately prescribed next plan
Blood counts NCCP 00280	Baseline blood count	Review results before treatment	Further counts if symptoms or clinical plan require	Review infection, bleeding and previous toxicity	Follow unresolved low counts
Kidneys, liver and fluids NCCP 00280	Renal/liver profile; verify clearance and dose suitability	Renal/liver tests before each cycle; hydration/electrolytes with cisplatin	Repeat sooner after dehydration or illness	Review organ function before continuation or modification	Follow kidney/electrolyte/liver abnormalities
Hearing and nerves NCCP 00280	Record baseline symptoms; audiology if indicated	Review symptoms before each cycle	Report tinnitus, hearing loss, numbness or loss of function	Balance benefit and lasting toxicity	Review persistent changes
Prevention and route NCCP 00280 · ANTIEMETIC	Confirm infusion or capsule plan and a written multi-day anti-sickness schedule	Check support before each treatment day; pharmacy confirms oral doses	Follow support through two days after last SACT; do not extend medicines yourself	Review interactions and support after any dose/route change	Confirm which support medicines stop or continue
Disease assessment NCCP 00280	Confirm extensive-stage SCLC and individual treatment aim	Assess symptoms and fitness	Extra assessment for new concerns	Scans/tests as directed by consultant; no universal scan interval	Agree follow-up or a separate next treatment plan

What each check is for

1 Before first treatment

Before dose authorisation

Confirm suitability and route.

Ask / act: Confirm the complete plan with the consultant and pharmacy.

Sources: NCCP 00280

2 Across Days 1–3

Every treatment day

The active medicines and route change within each cycle.

Ask / act: Confirm dose, route and today’s supportive medicines.

Sources: NCCP 00280

3 After the last treatment day

Two days after multi-day SACT

Prevention extends beyond the final dose.

Ask / act: Provide an individual written schedule and rescue instructions.

Sources: ANTIEMETIC

4 Course and response review

During and at the end of planned chemotherapy

Benefit and toxicity guide the next step.

Ask / act: Confirm completion, persistent-toxicity follow-up and any separate next regimen.

Sources: NCCP 00280

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-26 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.23